

COMMITTEE ON APPROPRIATIONS, TRANSPORTATION AND TECHNOLOGY
SENATE AMENDMENTS TO S.B. 1630
(Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.

[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 36, chapter 29, Arizona Revised Statutes, is
3 amended by adding article 3.1, to read:

4 ARTICLE 3.1. HOME AND COMMUNITY-BASED SERVICES FOR PERSONS
5 WITH SERIOUS MENTAL ILLNESS

6 36-2979. Definitions

7 IN THIS ARTICLE, UNLESS THE CONTEXT OTHERWISE REQUIRES:

8 1. "ADMINISTRATION" MEANS THE ARIZONA HEALTH CARE COST CONTAINMENT
9 SYSTEM ADMINISTRATION.

10 2. "BEHAVIORAL HEALTH RESIDENTIAL FACILITY" MEANS A HEALTH CARE
11 INSTITUTION THAT IS LICENSED PURSUANT TO THIS TITLE TO PROVIDE LEVEL I OR
12 II BEHAVIORAL HEALTH RESIDENTIAL SERVICES.

13 3. "HIGH-ACUITY SERIOUSLY MENTALLY ILL INDIVIDUAL" MEANS A PERSON
14 WHO MEETS A SERIOUS MENTAL ILLNESS [LONG-TERM] LEVEL OF CARE AND ONE OR
15 MORE PRIORITY CRITERIA LISTED IN SECTION 36-2979.04, SUBSECTION B.

16 4. "HOME AND COMMUNITY-BASED SERVICES":

17 (a) MEANS SERVICES AUTHORIZED UNDER ONE OR MORE MEDICAID STATE PLAN
18 AUTHORITIES, WAIVERS OR DEMONSTRATION AUTHORITIES, INCLUDING THOSE
19 AUTHORIZED UNDER 42 UNITED STATES CODE SECTION 1315 OR 1396n, THAT SUPPORT
20 INDIVIDUALS IN THE COMMUNITY AS AN ALTERNATIVE TO INSTITUTIONALIZATION.

21 (b) INCLUDES SERVICES THAT PROVIDE ASSISTANCE WITH ACTIVITIES OF
22 DAILY LIVING, MEDICATION ADMINISTRATION, SUPERVISION AND STRUCTURED
23 SUPPORTS COMPARABLE TO ASSISTED LIVING MODELS.

24 5. "MEMBER" MEANS A PERSON WHO IS ENROLLED IN THE PROGRAM.

25 6. "PROGRAM" MEANS THE HOME AND COMMUNITY-BASED SERVICE PROGRAM FOR
26 ADULTS WHO ARE SERIOUSLY MENTALLY ILL.

27 7. "QUALIFIED INCOME TRUST" MEANS A TRUST AS DESCRIBED IN 42 UNITED
28 STATES CODE SECTION 1396p(d)(4)(B).

1 8. "SERIOUSLY MENTALLY ILL" HAS THE MEANING PRESCRIBED IN SECTION
2 36-550.

3 9. "SERIOUS MENTAL ILLNESS [LONG-TERM] LEVEL OF CARE" MEANS THE
4 BEHAVIORAL HEALTH INSTITUTIONAL LEVEL OF CARE ADOPTED BY THE
5 ADMINISTRATION PURSUANT TO SECTION 36-2979.02.

6 36-2979.01. Home and community-based service program for
7 adults who are seriously mentally ill; request
8 for federal approval; stakeholder workgroup;
9 semiannual implementation updates

10 A. THE HOME AND COMMUNITY-BASED SERVICE PROGRAM FOR ADULTS WHO ARE
11 SERIOUSLY MENTALLY ILL IS ESTABLISHED. THE ADMINISTRATION SHALL DEVELOP
12 AND REQUEST AUTHORITY FROM THE CENTERS FOR MEDICARE AND MEDICAID SERVICES
13 TO IMPLEMENT A DEDICATED HOME AND COMMUNITY-BASED SERVICES BENEFIT FOR
14 ADULTS WHO ARE DETERMINED TO BE SERIOUSLY MENTALLY ILL.

15 B. [THROUGH THE SUBMISSION OF THE FIRST ANNUAL REPORT,] THE
16 ADMINISTRATION SHALL CONVENE A STAKEHOLDER WORKGROUP THAT INCLUDES
17 REPRESENTATIVES FROM BEHAVIORAL HEALTH PROVIDERS, FAMILY MEMBERS,
18 CAREGIVERS AND GUARDIANS OF INDIVIDUALS WHO ARE SERIOUSLY MENTALLY ILL,
19 COUNTIES, TRIBAL NATIONS AND COMMUNITY ORGANIZATIONS TO ADVISE ON PROGRAM
20 DESIGN AND IMPLEMENTATION. STAKEHOLDER INPUT SHALL SPECIFICALLY ADDRESS
21 THE NEEDS OF INDIVIDUALS WHO REQUIRE COMPLEX MEDICATION MANAGEMENT,
22 ENHANCED MONITORING AND STRUCTURED COMMUNITY-BASED SUPPORTS, INCLUDING
23 ASSISTED LIVING-TYPE MODELS, TO PROMOTE MEDICATION CONTINUITY AND SAFETY.

24 C. THE ADMINISTRATION SHALL REQUEST APPROVAL FROM THE CENTERS FOR
25 MEDICARE AND MEDICAID SERVICES FOR THE PROGRAM NOT LATER THAN JULY 1, 2027
26 AND SHALL BEGIN IMPLEMENTING THE PROGRAM NOT LATER THAN ONE YEAR AFTER THE
27 APPROVAL BY THE CENTERS FOR MEDICARE AND MEDICAID SERVICES.

28 D. UNTIL THE PROGRAM IS IMPLEMENTED, THE ADMINISTRATION SHALL
29 PROVIDE [QUARTERLY] [SEMIANNUAL] IMPLEMENTATION UPDATES TO THE PRESIDENT
30 OF THE SENATE, THE SPEAKER OF THE HOUSE OF REPRESENTATIVES AND THE
31 CHAIRPERSONS OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN
32 SERVICES COMMITTEES, OR THEIR SUCCESSOR COMMITTEES.

33 36-2979.02. Eligibility; financial eligibility; serious
34 mental illness long-term level of care;
35 criteria

36 A. TO BE ELIGIBLE FOR THE PROGRAM, AN INDIVIDUAL MUST MEET ALL OF
37 THE FOLLOWING:

38 1. HAVE BEEN DETERMINED TO BE SERIOUSLY MENTALLY ILL.

39 2. MEET THE SERIOUS MENTAL ILLNESS [LONG-TERM] LEVEL OF CARE
40 ADOPTED BY THE ADMINISTRATION PURSUANT TO SUBSECTION C OF THIS SECTION.

41 3. REQUIRE HOME AND COMMUNITY-BASED SERVICES IN ORDER TO AVOID
42 PLACEMENT IN A BEHAVIORAL HEALTH RESIDENTIAL FACILITY OR PSYCHIATRIC
43 INSTITUTION.

44 4. MEET THE FINANCIAL ELIGIBILITY REQUIREMENTS PURSUANT TO
45 SUBSECTION B OF THIS SECTION.

1 B. AN INDIVIDUAL WHO MEETS THE SERIOUS MENTAL ILLNESS [LONG-TERM]
2 LEVEL OF CARE ADOPTED BY THE ADMINISTRATION PURSUANT TO SUBSECTION C OF
3 THIS SECTION IS FINANCIALLY ELIGIBLE FOR THE PROGRAM IF THE INDIVIDUAL'S
4 INCOME DOES NOT EXCEED THREE HUNDRED PERCENT OF THE FEDERAL SUPPLEMENTAL
5 SECURITY INCOME BENEFIT RATE. AN INDIVIDUAL WHOSE INCOME EXCEEDS THIS
6 STANDARD MAY ESTABLISH A QUALIFIED INCOME TRUST TO ACHIEVE
7 ELIGIBILITY. RESOURCE AND SPOUSAL IMPOVERISHMENT STANDARDS SHALL BE THE
8 SAME AS REQUIRED PURSUANT TO ARTICLE 2 OF THIS CHAPTER FOR ENROLLEES IN
9 THE ARIZONA LONG-TERM CARE SYSTEM HOME AND COMMUNITY-BASED SERVICES.

10 C. THE ADMINISTRATION SHALL ADOPT A SERIOUS MENTAL ILLNESS
11 [LONG-TERM] LEVEL OF CARE SPECIFIC TO INDIVIDUALS WHO ARE SERIOUSLY
12 MENTALLY ILL THAT IS BASED ON BEHAVIORAL, FUNCTIONAL AND SAFETY CRITERIA,
13 WHICH MAY INCLUDE ANY OF THE FOLLOWING:

- 14 1. CURRENT OR RECENT COURT-ORDERED TREATMENT.
- 15 2. LEGAL GUARDIANSHIP DUE TO PSYCHIATRIC INCAPACITY.
- 16 3. RECENT DISCHARGE FROM A JAIL OR PRISON, THE STATE HOSPITAL OR A
17 BEHAVIORAL HEALTH RESIDENTIAL FACILITY.
- 18 4. REPEATED PSYCHIATRIC HOSPITALIZATIONS OR CRISIS EPISODES.
- 19 5. IMPAIRED JUDGMENT, DISORGANIZATION OR INABILITY TO PERFORM
20 ACTIVITIES OF DAILY LIVING DUE TO PSYCHIATRIC SYMPTOMS.
- 21 6. DOCUMENTED SAFETY RISKS, INCLUDING ELOPEMENT, FIRE OR WATER
22 MISUSE, AGGRESSION, DELUSIONAL BEHAVIOR OR EXPLOITATION RISK.
- 23 7. HOMELESSNESS OR IMMINENT RISK OF HOMELESSNESS.
- 24 8. HIGH-INTENSITY OR COMPLEX PSYCHOTROPIC MEDICATION REGIMENS
25 REQUIRING ENHANCED MONITORING TO ENSURE ADHERENCE AND TO IDENTIFY
26 POTENTIAL ADVERSE EFFECTS.
- 27 9. CLINICAL NEEDS THAT NECESSITATE STRUCTURED COMMUNITY-BASED
28 SUPPORTS, INCLUDING ASSISTED LIVING-TYPE SUPPORTS, TO MAINTAIN CONTINUITY
29 OF MEDICATION THERAPY, SUCH AS THOSE AT ELEVATED RISK OF RELAPSE,
30 DECOMPENSATION OR HOSPITALIZATION RELATED TO MEDICATION NONADHERENCE.

31 D. THE SERIOUS MENTAL ILLNESS [LONG-TERM] LEVEL OF CARE ADOPTED
32 PURSUANT TO SUBSECTION C OF THIS SECTION MAY NOT REQUIRE A NURSING
33 FACILITY LEVEL OF CARE OR PHYSICAL DISABILITY OR PHYSICAL IMPAIRMENT
34 CRITERIA AND MAY NOT USE A PREADMISSION SCREENING TOOL.

35 E. THE ADMINISTRATION MAY REFINE THE ASSESSMENT CRITERIA AND
36 PROCESSES THROUGH RULE OR POLICY, CONSISTENT WITH LEGISLATIVE INTENT,
37 INCLUDING ADJUSTMENT TO ASSESSMENT TOOLS, THRESHOLDS OR PROCESSES.

38 36-2979.03. Covered services: modification

39 A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID
40 SERVICES, THE ADMINISTRATION SHALL PROVIDE HOME AND COMMUNITY-BASED
41 SERVICES THAT ARE COMPARABLE TO THOSE AUTHORIZED UNDER ARTICLE 2 OF THIS
42 CHAPTER, WITH MODIFICATIONS APPROPRIATE FOR BEHAVIORAL HEALTH NEEDS. THE
43 HOME AND COMMUNITY-BASED SERVICES SHALL INCLUDE:

- 44 1. ATTENDANT CARE AND PERSONAL CARE.
- 45 2. HABILITATION WITH BEHAVIOR MANAGEMENT.
- 46 3. ADULT DAY HEALTH CARE WITH A BEHAVIORAL HEALTH FOCUS.

1 4. SUPERVISED COMMUNITY LIVING SERVICES, INCLUDING ASSISTED
2 LIVING-TYPE SUPPORTS.

3 5. RESPITE CARE.

4 6. HOME-DELIVERED MEALS.

5 7. NURSING, HOME HEALTH AND MEDICATION ADMINISTRATION SERVICES.

6 8. NONEMERGENCY TRANSPORTATION.

7 B. THE ADMINISTRATION SHALL ESTABLISH SERVICE DESCRIPTIONS AND
8 SCOPE AND STAFFING STANDARDS THROUGH RULE OR POLICY.

9 C. HOME AND COMMUNITY-BASED SERVICES MAY BE DELIVERED IN ANY
10 RESIDENTIAL SETTING AUTHORIZED UNDER STATE LAW WHEN AN INDIVIDUAL IS
11 SUBJECT TO A VALID COURT ORDER, GUARDIANSHIP OR INVOLUNTARY TREATMENT
12 AUTHORITY PURSUANT TO THIS TITLE, IF:

13 1. MEDICAID REIMBURSEMENT UNDER THIS ARTICLE IS LIMITED TO COVERED
14 HOME AND COMMUNITY-BASED SERVICES AND DOES NOT INCLUDE ROOM, BOARD,
15 SUPERVISION FOR CUSTODY OR ENFORCEMENT OF COURT ORDERS.

16 2. THE INDIVIDUAL IS AFFORDED PERIODIC REVIEW AND PLANNING FOR
17 TRANSITION TO A LESS RESTRICTIVE SETTING WHEN CLINICALLY APPROPRIATE.

18 D. THE ADMINISTRATION MAY ADD, MODIFY OR COMBINE SERVICES
19 CONSISTENT WITH FEDERAL APPROVAL AND LEGISLATIVE INTENT.

20 36-2979.04. Program capacity; enrollment factors; emergency
21 placements

22 A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID
23 SERVICES, ~~[ENROLLMENT IN THE PROGRAM IS LIMITED TO FIVE HUNDRED MEMBERS]~~
24 ~~[THE ADMINISTRATION SHALL IMPLEMENT THE PROGRAM FOR UP TO TWO HUNDRED~~
25 ~~FIFTY MEMBERS STATEWIDE]~~, INCLUDING TEMPORARY EMERGENCY PLACEMENTS. ~~[THE~~
26 ~~ADMINISTRATION MAY INCREASE ENROLLMENT ABOVE TWO HUNDRED FIFTY MEMBERS IF~~
27 ~~THE ADMINISTRATION DEMONSTRATES, THROUGH DATA REPORTED PURSUANT TO SECTION~~
28 ~~36-2979.06, REDUCED UTILIZATION OF HIGH-COST SERVICES OR COST AVOIDANCE IN~~
29 ~~STATE-FUNDED SYSTEMS, SUBJECT TO REVIEW BY THE JOINT LEGISLATIVE BUDGET~~
30 ~~COMMITTEE. ENROLLMENT MAY NOT EXCEED ONE THOUSAND MEMBERS UNLESS~~
31 ~~AUTHORIZED BY THE LEGISLATURE.]~~

32 B. WHEN CAPACITY IS LIMITED, THE ADMINISTRATION SHALL ASSIGN
33 AVAILABLE SLOTS BASED ON THE FOLLOWING ~~[ORDER OF PRIORITY]~~:

34 1. INDIVIDUALS UNDER COURT-ORDERED TREATMENT.

35 2. INDIVIDUALS WITH LEGAL GUARDIANSHIP DUE TO PSYCHIATRIC
36 INCAPACITY.

37 3. INDIVIDUALS DISCHARGED FROM A JAIL OR PRISON, THE STATE HOSPITAL
38 OR A BEHAVIORAL HEALTH RESIDENTIAL FACILITY.

39 4. INDIVIDUALS WHO ARE HOMELESS OR AT IMMINENT RISK OF
40 HOMELESSNESS.

41 5. INDIVIDUALS WITH REPEATED CRISIS EPISODES, PSYCHIATRIC
42 HOSPITALIZATIONS OR PUBLIC SAFETY INVOLVEMENT.

43 6. INDIVIDUALS PRESENTING SIGNIFICANT SAFETY RISKS DUE TO
44 PSYCHIATRIC SYMPTOMS.

45 7. INDIVIDUALS WITH HIGH-INTENSITY OR COMPLEX PSYCHOTROPIC
46 MEDICATION REGIMENS REQUIRING ENHANCED MONITORING TO ENSURE ADHERENCE AND
47 TO IDENTIFY POTENTIAL ADVERSE EFFECTS.

1 C. A COURT MAY RECOMMEND PARTICIPATION IN THE PROGRAM BUT MAY NOT
2 COMPEL THE ADMINISTRATION TO EXCEED THE ENROLLMENT CAP.

3 D. THE ADMINISTRATION MAY RESERVE A PORTION OF AVAILABLE CAPACITY
4 FOR EMERGENCY OR PRIORITY PLACEMENTS.

5 36-2979.05. Provider requirements; enhanced reimbursement
6 rates

7 A. PROVIDERS SHALL DOCUMENT BEHAVIORAL INTERVENTIONS, CRISIS
8 SUPPORTS AND STAFFING ADJUSTMENTS BEFORE INITIATING DISCHARGE.

9 B. A PROVIDER THAT RECEIVES REIMBURSEMENT UNDER THE PROGRAM SHALL
10 IMPLEMENT EVICTION-PREVENTION PROTOCOLS AND OBTAIN REGIONAL BEHAVIORAL
11 HEALTH AUTHORITY APPROVAL BEFORE ISSUING NONEMERGENCY NOTICES TO VACATE.

12 C. THE ADMINISTRATION MAY ADOPT ENHANCED REIMBURSEMENT RATES FOR
13 HIGH-ACUITY SERIOUSLY MENTALLY ILL INDIVIDUALS WHO ARE RECEIVING HOME AND
14 COMMUNITY-BASED SERVICES.

15 D. THIS SECTION DOES NOT LIMIT THE ADMINISTRATION'S AUTHORITY TO
16 ENSURE THE HEALTH AND SAFETY OF PARTICIPANTS.

17 E. THIS SECTION DOES NOT REQUIRE A PROVIDER TO CONTINUE SERVICES
18 WHEN IMMEDIATE AND DOCUMENTED RISKS TO HEALTH OR SAFETY CANNOT BE
19 MITIGATED THROUGH REASONABLE CLINICAL INTERVENTIONS.

20 36-2979.06. Annual report

21 BEGINNING ONE YEAR AFTER PROGRAM IMPLEMENTATION AND EACH YEAR
22 THEREAFTER, THE ADMINISTRATION SHALL SUBMIT A REPORT TO THE GOVERNOR, THE
23 PRESIDENT OF THE SENATE AND THE SPEAKER OF THE HOUSE OF REPRESENTATIVES
24 THAT INCLUDES:

25 1. THE NUMBER OF ENROLLED MEMBERS AND THE NUMBER OF INDIVIDUALS WHO
26 ARE ON THE WAITLIST.

27 2. THE PERCENTAGE OF MEMBERS WHO ARE UNDER COURT-ORDERED TREATMENT,
28 GUARDIANSHIP OR CRIMINAL JUSTICE SUPERVISION.

29 3. HOUSING STABILITY OUTCOMES FOR MEMBERS.

30 4. HOSPITALIZATION, CRISIS SERVICE[,] ~~[AND]~~ JAIL[, EMERGENCY
31 DEPARTMENT, EVALUATION AGENCY AND SCREENING AGENCY] UTILIZATION OF
32 MEMBERS.

33 5. PROVIDER DENIALS AND DISCHARGES AND THE REASONS FOR THE DENIALS
34 AND DISCHARGES OF MEMBERS.

35 6. THE FISCAL IMPACT OF AND ESTIMATED COST AVOIDANCE RELATED TO THE
36 PROGRAM.

37 36-2979.07. Authority to seek and maintain federal approval;
38 rules

39 A. THE ADMINISTRATION SHALL SEEK AND MAINTAIN ANY NECESSARY FEDERAL
40 APPROVALS AND MAY OPERATE THE PROGRAM PURSUANT TO ONE OR MORE APPROVED
41 MEDICAID STATE PLAN AUTHORITIES, WAIVERS OR DEMONSTRATION AUTHORITIES,
42 INCLUDING THOSE AUTHORIZED UNDER 42 UNITED STATES CODE SECTION 1315 OR
43 1396n.

44 [B. IF THE CENTERS FOR MEDICARE AND MEDICAID SERVICES DENIES OR
45 DOES NOT APPROVE A REQUEST NECESSARY TO IMPLEMENT THIS ARTICLE, THE
46 ADMINISTRATION, TO THE EXTENT ALLOWED BY FEDERAL LAW, SHALL CONTINUE TO
47 PURSUE APPROVAL THROUGH MODIFICATIONS, RESUBMISSIONS OR ALTERNATIVE

1 FEDERAL AUTHORITY AND SHALL REPORT THE STATUS OF THESE EFFORTS IN THE
2 SEMIANNUAL IMPLEMENTATION UPDATES REQUIRED PURSUANT TO SECTION
3 36-2979.01.]

4 [B.] [C.] THE ADMINISTRATION [SHALL] [MAY] ADOPT RULES TO
5 IMPLEMENT THIS ARTICLE.

6 [C.] [D.] THIS ARTICLE DOES NOT EXPAND OR MODIFY STANDARDS FOR
7 INVOLUNTARY TREATMENT UNDER THIS TITLE.

8 Enroll and engross to conform

9 Amend title to conform

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